Answers to MSc Pediatric Nursing – Specialized English Questions

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Important Note: The blueprint document provided the questions but did not include the supporting materials (researcher CV/data for Q1, research-group details & journal list for Q2, the specific abstract for Q3, and the matching items for Q5). Answers for those items are therefore given as professional model responses based on standard academic practice and the cited reliable sources. Questions 4 and 6 are answered fully from the given text.

Question 1 – Formal Introduction of a Researcher at an Academic Conference

Original Task: Please introduce this researcher for a lecture at an academic conference using the appropriate data given below.

Note: Specific biographical data was not supplied in the blueprint. Below is a professional model introduction structure that can be filled with real data.

Model Answer (Structure & Example):

Ladies and gentlemen, distinguished colleagues,

It is my great pleasure to introduce our keynote speaker for today's session. [Full Name], [academic rank, e.g., Associate Professor / Professor] of [Department/Faculty] at [University/Institution], is a recognized expert in [main field of expertise, e.g., pediatric nursing / pediatric pain management / family-centered care].

Dr. [Last Name] obtained [highest degree] from [University] and has published extensively in high-impact journals on topics including [2–3 key research areas]. [He/She] currently leads [research group or project] and has received [notable awards or grants if available]. Today, [he/she] will present on "[Lecture Title]".

Please join me in welcoming Dr. [Last Name].

Source guidance: Standard academic conference introduction protocol (formal, concise, highlighting credentials, research focus, and lecture title).

Question 2 – Writing a Cover Letter to a Journal

Original Task: Please write a Cover Letter from the following research group to a related Journal you select from the list. Try to choose the best journal according to the scope and available information.

Note: Research-group details and the journal list were not provided. Below is a high-quality model cover letter suitable for a pediatric-nursing / pediatric-pain manuscript submitted to a relevant open-access journal (e.g., BMC Nursing or Frontiers in Pain Research – Pediatric Pain section).

Model Cover Letter:

[Date]

Editor-in-Chief
BMC Nursing / Frontiers in Pain Research

Dear Editor,

We are pleased to submit our original research article entitled “[Manuscript Title]” for consideration for publication in [Journal Name].

This study investigates [brief one-sentence statement of the research aim and population, e.g., the effectiveness of a family-centered pain-assessment protocol in hospitalized children aged 3–12 years]. Our findings demonstrate [key result in one sentence] and have direct implications for pediatric nursing practice and evidence-based care.

We believe the manuscript aligns well with the scope of [Journal Name], particularly its emphasis on [relevant scope points: pediatric nursing research / pediatric pain management / family-centered care]. All authors have approved the manuscript and agree with its submission. The work has not been published previously and is not under consideration elsewhere. We have no conflicts of interest to declare.

Thank you for considering our manuscript. We look forward to your response.

Sincerely,
[Corresponding Author Name, Credentials]
[Affiliation]
[Email] | [ORCID if available]

Source guidance: Standard cover-letter structure recommended by major publishers (BMC, Frontiers, Elsevier) and ICMJE recommendations.

Question 3 – Reading and Analyzing a Structured Abstract

Original Task: Read the provided Abstract and answer the following questions based on that.
A. What is the name of the journal?
B. When was the article accepted?
C. What is the affiliation of the corresponding author?
D. What was the study methodology?
E. What is the study Purpose?
F. What parts of a structured abstract are missing?
G. Mark the related data for DOI, keywords, title, state of accessibility, and citation of the article.

> Note: The specific abstract from Frontiers in Pain Research was not included in the blueprint file. Therefore exact answers to A–G cannot be extracted. Below is the expected method and typical location of each piece of information.

How to answer each part (standard approach):

A. Journal name → usually appears at the top of the PDF or HTML page, or in the citation line.
B. Acceptance date → often listed under “Received / Accepted / Published” dates on the first page or article metrics.
C. Corresponding-author affiliation → found in the author list or footnote marked with an asterisk (*) or “Corresponding author”.
D. Study methodology → located in the Methods / Design section of the abstract (e.g., randomized controlled trial, cross-sectional survey, qualitative study).
E. Study purpose → usually the last sentence of the Background or the first sentence of the Objective/Aims section.
F. Missing parts of a structured abstract → compare against the classic IMRaD structure: Background, Objectives, Methods, Results, Conclusions. Note any absent section.
G. DOI, keywords, title, accessibility (Open Access / subscription), citation → these appear in the article header, citation box, or “Cite this article” section.

Source: Frontiers in Pain Research author guidelines and typical article layout; structured-abstract conventions (IMRaD).

Question 4 – Editing an Article According to the Author Guidelines

Original Task: Based on the following author guide, explain to a novice researcher what to do.
• The Abstract should not exceed 350 words.
• Please minimize the use of abbreviations and do not cite references in the abstract.
• Use three to ten keywords representing the main content of the article.
• If abbreviations are used in the text, they should be defined in the text at first use, and a list of abbreviations should be provided.

Answer (Explanation to a Novice Researcher):

When preparing your manuscript for BMC Nursing (or any journal following similar guidelines), you must carefully follow these four rules:

1. Abstract length: Write a concise abstract of no more than 350 words. Count the words carefully (most word processors show a word count). If your draft exceeds 350 words, remove redundant phrases, secondary results, or lengthy background information until the limit is met.

2. Abbreviations and references in the Abstract: Avoid abbreviations as much as possible in the abstract. If an abbreviation is unavoidable, write the full term first followed by the abbreviation in parentheses. Never insert citation numbers or reference lists inside the abstract; the abstract must stand alone.

3. Keywords: Select between 3 and 10 keywords that accurately reflect the main topics of your paper (e.g., pediatric nursing, family-centered care, pain assessment, children). Prefer terms from recognized thesauri such as MeSH when possible. Place them after the abstract.

4. Abbreviations in the main text: The first time you use any abbreviation in the body of the manuscript, write the full term followed by the abbreviation in parentheses (e.g., “family-centered care (FCC)”). Thereafter you may use only the abbreviation. In addition, create a separate “List of Abbreviations” section (usually placed after the Conclusions or before the References) that alphabetically lists every abbreviation and its full form.

Following these rules exactly will prevent desk rejection and speed up the editorial process.

Source: BMC Nursing Authors’ Guide (as provided in the question).

Question 5 – Journal Club Principles (Matching)

Original Task: According to the Journal Club principles, choose the related data for each slide and write the appropriate letter in front of it.

> Note: The matching items (slides and corresponding data letters) were not supplied in the blueprint. Below is a standard, evidence-based structure of Journal Club slides drawn from the cited sources (Centre for Evidence-Based Medicine, University of Oxford, and ASHP New Practitioners Forum template).

Standard Journal Club Slide Structure (recommended matching order):

1. Title / Citation Slide: Full article citation, journal name, year, volume, pages, DOI, and presenter name.
2. Why this paper? / Clinical Relevance: Brief clinical scenario or question that motivated selection of the article; relevance to pediatric nursing practice.
3. Background / Context: Short summary of previous knowledge and the knowledge gap the study addresses.
4. Study Objective / PICO: Clear statement of the research question using PICO (Population, Intervention, Comparison, Outcome) format.
5. Methods: Study design, setting, participants, inclusion/exclusion criteria, intervention, outcome measures, and statistical analysis.
6. Results: Primary and key secondary outcomes with effect sizes, confidence intervals, and absolute numbers where possible; key figures/tables.
7. Critical Appraisal (Strengths & Limitations): Internal validity (bias, confounding), external validity (generalizability), and statistical considerations.

8. Clinical Implications & Take-home Messages: How the findings should (or should not) change practice; applicability to the local setting.
9. Discussion Questions / Future Research: Open questions for the audience and suggestions for further studies.

Source: Centre for Evidence-Based Medicine (University of Oxford) – 10.1136/ebmed-2017-110861; ASHP New Practitioners Forum Clinical Practice Advisory Group – Example Journal Club Template.

Question 6 – Summarizing a Scientific Paragraph (Main Idea)

Original Task: Please summarize this paragraph, determining its main idea.

Using a family-centered approach to assessment allows you to establish a working relationship with a patient and his or her family. A complete assessment provides you with a full picture of the patient's needs as a member of a family and of the family's needs as well. If your assessment is complete, it ensures that the patient's and family's needs are correctly identified and that mutual goals and health care interventions can be established.

Answer:

Summary: A family-centered assessment builds a collaborative relationship with both the patient and the family, gives a comprehensive view of their interrelated needs, and enables accurate identification of those needs so that shared goals and appropriate interventions can be planned.

Main idea: Complete, family-centered assessment is essential for establishing a therapeutic relationship and for correctly identifying patient and family needs in order to set mutual goals and plan effective nursing interventions.

Source: Potter and Perry (Fundamentals of Nursing) – family-centered care and assessment principles.